

Policy: Documentation of Surgery-Related Policies and Procedures — Ophthalmology

Field	Value
Policy number	OPH-SURG-001
Policy owner	Chief of Ophthalmology / Ophthalmic Surgical Services Director
Approving body	Medical Executive Committee; Surgical Services Committee
Applies to	All ophthalmic operating rooms, ambulatory surgery centers (ASC), procedure rooms, and in-office surgical suites operated by the organization
Effective date	<i>to be set at approval</i>
Last reviewed	<i>to be set at approval</i>
Review cycle	Annually, or sooner upon regulatory change, adverse event, or change in scope of service

Adaptation notice. This document is a template. Before adoption it must be reviewed and amended by the organization's clinical leadership, infection prevention, pharmacy, risk management, and legal/compliance functions so that it reflects local scope of practice, applicable state and national regulation, accreditation standards, and payer requirements. Where this policy conflicts with a law, regulation, or accreditation standard binding on the organization, that external requirement prevails.

1. Purpose

To establish the requirement that all surgery-related policies and procedures governing ophthalmic surgical care are **written, approved, accessible, current, and auditable**, and to define the minimum set of topics that must be covered. Documented policy is the mechanism by which the organization makes ophthalmic surgical practice consistent across surgeons, sites, and shifts; supports the prevention of never events such as wrong-site, wrong-patient, wrong-implant surgery; and demonstrates compliance to regulators, accreditors, and payers.

2. Scope

This policy applies to every surgical or invasive ophthalmic procedure performed under the organization's authority, regardless of setting or anesthesia type, including but not limited to:

- Cataract extraction with or without intraocular lens (IOL) implantation
- Corneal procedures (keratoplasty, DSAEK/DMEK, cross-linking, pterygium excision)
- Glaucoma surgery (trabeculectomy, tube shunt, MIGS)
- Vitreoretinal surgery (vitrectomy, scleral buckle, retinal detachment repair)
- Intravitreal injections and other office-based invasive procedures
- Oculoplastic and orbital surgery (ptosis repair, blepharoplasty, dacryocystorhinostomy, enucleation, evisceration)

- Strabismus surgery
- Refractive surgery (LASIK, PRK, phakic IOL)
- Laser procedures (YAG capsulotomy, laser peripheral iridotomy, panretinal photocoagulation, SLT)

It applies to all personnel involved in those procedures: attending ophthalmologists, fellows, residents, anesthesia professionals, registered nurses, surgical technologists, ophthalmic technicians, sterile processing staff, and contracted or locum personnel.

3. Definitions

Term	Definition
Procedural document	A written policy, procedure, protocol, standard work instruction, checklist, or preference card that directs clinical or operational practice.
Operative eye	The eye designated for surgery, documented as "right eye (OD)" or "left eye (OS)" in words, never by abbreviation alone in the consent.
Universal Protocol	The pre-procedure verification, site marking, and time-out process required to prevent wrong-site, wrong-procedure, and wrong-person surgery.
Implant record	The permanent record of any device placed in or on the eye, including IOLs, glaucoma drainage devices, scleral buckles, punctal plugs, and orbital implants.
TASS	Toxic Anterior Segment Syndrome — acute sterile postoperative inflammation, typically caused by contaminants on instruments or in intraocular solutions.
Controlled document	A document under version control, with an owner, approval record, effective date, and scheduled review date.

4. Policy statements

4.1 Documentation is mandatory

Every surgery-related practice that affects patient safety, clinical outcome, regulatory compliance, or billing integrity in ophthalmology **shall** be captured in a controlled document. Undocumented custom or individual surgeon preference is not a substitute for policy. Where a surgeon's technique varies legitimately from the standard, that variation shall be recorded on an approved surgeon preference card, which is itself a controlled document.

4.2 Minimum required topic set

The Ophthalmic Surgical Services Director shall maintain documented policies and procedures covering, at minimum, every topic listed in Section 6. An annual gap analysis against that inventory is required.

4.3 Control and accessibility

Each procedural document shall carry a document number, version, owner, approval body, approval date, effective date, and next review date. The current approved version shall be accessible to all staff at the point of care — electronically at every ophthalmic OR workstation, and in a readable offline copy available during network or power failure. Superseded versions shall be withdrawn from circulation and retained in an archive per the organization's record-retention schedule.

4.4 Review and revision

Documents shall be reviewed at least annually. Review shall be triggered ahead of schedule by any of: a relevant adverse event or near miss; a change in law, regulation, or accreditation standard; introduction of a new procedure, device, implant, or drug; a change in physical site or scope of service; or an internal or external audit finding.

4.5 Education and attestation

Staff shall be educated on new and materially revised documents before the effective date. Attestation of review shall be recorded for each person whose practice the document governs, and retained in the training record. Onboarding of new personnel, including locums and contracted staff, shall include the ophthalmic surgical document set relevant to their role.

4.6 Auditability

Compliance with documented procedures shall be verified through scheduled audit — at minimum quarterly observational audit of the Universal Protocol and time-out, and quarterly audit of sterilization and implant records. Audit findings, corrective actions, and re-audit results shall be reported to the Surgical Services Committee.

5. Responsibilities

Role	Responsibility
Chief of Ophthalmology	Owns the document set; ensures clinical content is current and evidence-based; chairs or delegates the annual gap analysis.
Ophthalmic Surgical Services Director	Maintains the document inventory, version control, review calendar, and distribution; escalates overdue reviews.
Medical Executive Committee	Approves clinical policies and material revisions.
Infection Prevention	Approves sterilization, reprocessing, TASS-prevention, and environmental cleaning content.
Pharmacy	Approves ophthalmic medication, compounding, intracameral/intravitreal drug, and look-alike/sound-alike content.
Risk Management / Quality	Owns adverse-event reporting, root cause analysis, and audit reporting.
Operating surgeon	Complies with documented procedure; performs and attests to verification steps; completes the operative record.
Perioperative nursing and surgical technologists	Execute documented procedures; complete counts, implant logs, and time-out documentation.
All staff	Stop-the-line authority: any team member may halt a procedure when a required verification step is incomplete or a discrepancy is unresolved, without fear of reprisal.

6. Required policy and procedure inventory

The following topics **must** each be covered by at least one current, approved, controlled document. This table is the checklist used for the annual gap analysis.

6.1 Patient selection, consent, and scheduling

#	Required document topic
6.1.1	Indications, contraindications, and criteria for each ophthalmic procedure offered
6.1.2	Informed consent: content, who may obtain it, documentation of laterality in words, risks/benefits/alternatives, interpreter use, and consent for anesthesia
6.1.3	Consent and assent for minors; consent for patients lacking decision-making capacity; surrogate decision-makers
6.1.4	Scheduling and booking, including how laterality and implant requirements are captured in the schedule
6.1.5	Patient selection criteria for the ASC/office setting versus hospital setting, including ASA class limits and escalation criteria
6.1.6	Cancellation, rescheduling, and same-day case postponement

6.2 Preoperative assessment and preparation

#	Required document topic
6.2.1	History and physical: content, timing, and required update on the day of surgery
6.2.2	Preoperative ophthalmic measurement: biometry, keratometry, topography, and IOL power calculation, including required second-check of calculations
6.2.3	Anticoagulant, antiplatelet, and alpha-blocker (floppy iris) management
6.2.4	NPO status, diabetes and insulin management, and home medication instruction
6.2.5	Preoperative dilation, ophthalmic drop administration, and periocular antisepsis
6.2.6	Allergy, latex, and adverse drug reaction screening
6.2.7	Pregnancy screening where clinically indicated
6.2.8	Preoperative laboratory and diagnostic testing criteria (including the standard that routine testing is not required for low-risk cataract surgery absent clinical indication)

6.3 Wrong-site, wrong-patient, wrong-implant prevention

#	Required document topic
6.3.1	Universal Protocol: pre-procedure verification, site marking, and time-out, adapted to ophthalmology
6.3.2	Surgical site marking: who marks, when, with what, placement on the ipsilateral brow or forehead, visibility after draping, and handling of patient refusal
6.3.3	Time-out content: patient identity using two identifiers, procedure, operative eye, implant type and power, anesthesia plan, allergies, antibiotic and anticoagulation status, and required team participation
6.3.4	Independent verification of IOL model, power, and laterality against the biometry source document before the lens is opened, with two-person confirmation
6.3.5	Verification for bilateral and sequential same-day procedures, including separation of instrument sets, drug lots, and implants between eyes
6.3.6	Discrepancy resolution and stop-the-line escalation
6.3.7	Laser procedure verification, including treatment eye, laser parameters, and settings confirmation

6.4 Anesthesia and sedation

#	Required document topic
6.4.1	Anesthesia options by procedure: topical, sub-Tenon's, peribulbar, retrobulbar, regional block, monitored anesthesia care, and general anesthesia
6.4.2	Pre-anesthesia evaluation and anesthesia plan documentation
6.4.3	Block technique, who may perform it, block site verification, and block-related complication management (globe perforation, retrobulbar hemorrhage, brainstem anesthesia)
6.4.4	Intraoperative monitoring standards and required documentation intervals
6.4.5	Sedation levels, rescue capability, and reversal agent availability
6.4.6	Malignant hyperthermia preparedness where volatile agents or succinylcholine are available
6.4.7	Post-anesthesia recovery, discharge criteria, and responsible-adult escort requirement

6.5 Intraoperative practice

#	Required document topic
6.5.1	Surgical attire, hand hygiene, surgical scrub, gowning, gloving, and traffic control in the ophthalmic OR
6.5.2	Patient positioning, draping, and airway/oxygen management under drapes, including fire risk with supplemental oxygen
6.5.3	Surgical fire prevention and response, including cautery and laser use near oxygen
6.5.4	Sponge, sharp, instrument, and small-item counts appropriate to microsurgery
6.5.5	Specimen handling, labeling, and transport (corneal buttons, biopsies, explanted devices)
6.5.6	Intraoperative medication management: labeling of all solutions on the sterile field, intracameral antibiotic preparation, dilating and viscoelastic agents, and prevention of intraocular injection of the wrong agent
6.5.7	Management of intraoperative complications: posterior capsule rupture, vitreous loss, suprachoroidal hemorrhage, dropped nucleus, and conversion of planned procedure
6.5.8	Equipment failure contingency: phacoemulsification unit, microscope, vitrector, and laser backup plans
6.5.9	Operative report content and timing, including a written or dictated note before the patient leaves the procedural area

6.6 Implants and devices

#	Required document topic
6.6.1	Implant record: manufacturer, model, serial/lot number, power, eye implanted, expiration, and placement in the permanent record and patient-facing implant card
6.6.2	Implant inventory, storage, expiration checks, and consignment handling
6.6.3	Implant recall management and patient notification
6.6.4	Explant handling, documentation, and return to manufacturer
6.6.5	Introduction of new devices or implants: evaluation, approval, training, and credentialing requirements before first use
6.6.6	Unique Device Identifier capture where required

6.7 Infection prevention, sterilization, and TASS

#	Required document topic
6.7.1	Instrument cleaning, decontamination, and reprocessing specific to ophthalmic microsurgical instruments, including lumened handpieces
6.7.2	Sterilization method, load documentation, biological and chemical indicator monitoring, and release criteria
6.7.3	Immediate-use steam sterilization: restricted indications, documentation, and prohibition as a routine substitute for adequate inventory
6.7.4	Single-use device policy and prohibition of unvalidated reuse
6.7.5	TASS prevention bundle: detergent and water quality, rinse volumes, enzymatic cleaner use, ophthalmic viscosurgical device residue, and cluster investigation and reporting procedure
6.7.6	Endophthalmitis prophylaxis: povidone-iodine antiseptics, draping of lashes, and intracameral antibiotic protocol where adopted
6.7.7	Surgical site infection surveillance, endophthalmitis case definition, reporting, and response
6.7.8	Environmental cleaning between cases and terminal cleaning; air handling, temperature, and humidity requirements
6.7.9	Bloodborne pathogen exposure, sharps safety, and post-exposure management
6.7.10	Staff and patient communicable disease screening and transmission-based precautions

6.8 Postoperative care

#	Required document topic
6.8.1	Immediate postoperative monitoring, eye shield and patching, and discharge criteria
6.8.2	Written discharge instructions, including activity restriction, drop regimen, shield use, and warning signs requiring urgent contact
6.8.3	Follow-up schedule by procedure, including day-one review requirements and who may perform them
6.8.4	24-hour on-call coverage and patient access to an ophthalmologist after hours
6.8.5	Management of postoperative complications: elevated intraocular pressure, wound leak, corneal edema, suspected endophthalmitis, retinal detachment, and unplanned return to the operating room
6.8.6	Emergency transfer agreement and transfer procedure from the ASC or office setting to an acute care hospital
6.8.7	Postoperative refraction and spectacle prescription timing

6.9 Personnel, competency, and credentialing

#	Required document topic
6.9.1	Credentialing and privileging criteria by ophthalmic procedure, including volume and outcome criteria where adopted
6.9.2	Proctoring and competency validation for new privileges and new technology
6.9.3	Resident and fellow supervision levels by procedure and training stage, with documentation of attending presence
6.9.4	Role-specific competency assessment and annual skills validation for nursing, technologist, and sterile processing staff
6.9.5	Ongoing and focused professional practice evaluation
6.9.6	Industry representative presence in the OR: approval, credentialing, and scope of involvement

6.10 Quality, safety, and records

#	Required document topic
6.10.1	Adverse event, near miss, and sentinel event reporting; root cause analysis; and corrective action tracking
6.10.2	Never-event response, including disclosure to the patient and family
6.10.3	Ophthalmic surgical quality indicators and their review cadence (for example: unplanned return to OR, posterior capsule rupture rate, endophthalmitis rate, refractive outcome accuracy, case cancellation rate, time-out compliance)
6.10.4	Medical record content and completion timelines for the operative record
6.10.5	Record retention, amendment, and release of information
6.10.6	Photography, video recording, and imaging in the OR, including consent and storage
6.10.7	Emergency preparedness, power failure, and downtime procedures affecting surgical services
6.10.8	Patient complaint and grievance handling related to surgical care

7. Procedure — maintaining the documented set

1. **Inventory.** The Ophthalmic Surgical Services Director maintains a register of all controlled documents in scope, listing document number, title, owner, version, approval date, effective date, and next review date.
2. **Gap analysis.** At least annually, the register is compared against Section 6. Every topic without a current approved document is logged as a gap with a named owner and a due date.
3. **Drafting and revision.** The content owner drafts or revises the document. Any document touching sterilization, medication, or anesthesia is routed to Infection Prevention, Pharmacy, or Anesthesia respectively for concurrence before approval.
4. **Approval.** Clinical documents are approved by the Medical Executive Committee; operational documents by the Surgical Services Committee. Approval is recorded with date and approving body.
5. **Publication.** The approved version is published to the point-of-care repository, the prior version is withdrawn and archived, and the register is updated.

6. **Education.** Staff affected by a new or materially revised document are educated before the effective date, and attestations are recorded.
7. **Audit.** Compliance is audited on the schedule in Section 4.6. Findings go to the Surgical Services Committee with corrective actions and re-audit dates.
8. **Escalation.** A document more than 30 days past its review date is reported to the Chief of Ophthalmology; more than 90 days past, to the Medical Executive Committee.

8. Compliance and enforcement

Failure to follow an approved surgical policy or procedure is managed under the organization's professional conduct and corrective action processes. Repeated non-compliance with verification procedures — particularly the Universal Protocol, implant verification, or count procedures — is reviewed through the professional practice evaluation process and may affect privileges. Reporting a near miss or an error in good faith is protected and shall not itself be a basis for corrective action.

9. Related documents

- Organization-wide Universal Protocol policy
- Organization-wide Informed Consent policy
- Infection Prevention Plan and Sterile Processing manual
- Medication Management policy and Pharmacy formulary
- Emergency Transfer Agreement (ASC sites)
- Credentialing and Privileging manual, ophthalmology privilege delineation
- Surgeon preference cards (by surgeon and procedure)

10. References

External standards and guidance to be cited in the locally adopted version, verified against the edition current at approval:

- The Joint Commission — Universal Protocol for Preventing Wrong Site, Wrong Procedure, Wrong Person Surgery; and applicable Ambulatory Care or Hospital accreditation standards
- Centers for Medicare & Medicaid Services — Conditions for Coverage for Ambulatory Surgical Centers (42 CFR Part 416) and Conditions of Participation for Hospitals (42 CFR Part 482)
- American Academy of Ophthalmology — Preferred Practice Pattern guidelines (Cataract in the Adult Eye, and others relevant to procedures offered) and patient safety statements
- American Society of Cataract and Refractive Surgery / ASCRS–ASORN — Recommended Practices for Cleaning and Sterilizing Intraocular Surgical Instruments (TASS prevention)
- AORN — Guidelines for Perioperative Practice
- Association for the Advancement of Medical Instrumentation — sterilization standards (ANSI/AAMI ST79)
- Accreditation Association for Ambulatory Health Care standards, where applicable
- Applicable state licensure regulations for surgical facilities and office-based surgery

Appendix A — Annual gap analysis worksheet

Copy this table and complete one row per topic in Section 6.

Topic #	Topic	Covering document (number & version)	Approved date	Next review	Status (Current / Overdue / Gap)	Owner	Action & due date
6.1.1							
6.1.2							
...							

Appendix B — Ophthalmic time-out minimum content

The time-out is performed immediately before incision, with all team members present, active, and free of competing tasks. Verbal confirmation is required for each item:

1. Patient identity, confirmed with two identifiers
2. Procedure to be performed, stated in full
3. **Operative eye**, stated as right or left, confirmed against the consent, the site mark, and the source document — not against the schedule alone
4. Implant type, model, and power, confirmed against the biometry source document
5. Allergies
6. Anesthesia plan and any block already performed, including the eye blocked
7. Antibiotic prophylaxis given or intentionally omitted
8. Anticipated complications, special equipment, and backup device availability
9. Sterilization indicator verification for the instrument set in use
10. Explicit confirmation that all team members agree, and that anyone may stop the procedure if a discrepancy exists

Any unresolved discrepancy halts the procedure until the source document is retrieved and the discrepancy is resolved and documented.